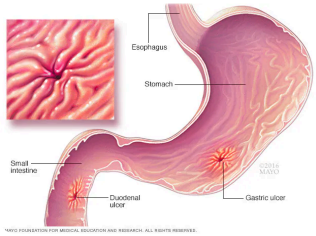
Peptic ulcer

Overview



Ulcers

Enlarge image

Peptic ulcers are open sores on the inner lining of the stomach and the upper part of the small intestine. The most common symptom of a peptic ulcer is stomach pain.

Peptic ulcers include:

- **Gastric ulcers**, which appear on the inside of the stomach.
- **Duodenal ulcers**, which appear on the inside of the upper part of the small intestine, called the duodenum.

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The most common causes of peptic ulcers are infection with the germ Helicobacter pylori (H. pylori) and long-term use of nonsteroidal anti-inflammatory drugs (NSAIDs). These include ibuprofen (Advil, Motrin IB, others) and naproxen sodium (Aleve).

Stress and spicy foods do not cause peptic ulcers. But they can make symptoms worse.

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Symptoms

Many people with peptic ulcers don't have symptoms. If there are symptoms, they may include:

- Dull or burning stomach pain. For some people, pain may be worse between meals and at night. For others, it may be worse after eating.
- Feeling of fullness or bloating.

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- Belching.
- Heartburn.
- Nausea.

Peptic ulcers can cause bleeding from the ulcer. Then symptoms might include:

- Vomiting blood, which may appear red or black.
- Having dark blood in stools, or stools that are black or tarry.
- Feeling dizzy or fainting.

When to see a doctor

See your healthcare professional if you're vomiting blood, having dark blood in stools or are feeling dizzy. Also see your healthcare professional if nonprescription antacids and acid blockers relieve your pain but the pain returns.

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Causes

Peptic ulcers happen when acid in the organs that food travels through, called the digestive tract, eats away at the inner surface of the stomach or small intestine. The acid can create a painful open sore that may bleed.

Your digestive tract is coated with a mucous layer that most often protects against acid. But if the amount of acid increases or the amount of mucus decreases, you could develop an ulcer.

Common causes include:

- **Helicobacter pylori.** This germ lives in the mucous layer that covers and protects tissues that line the stomach and small intestine. The H. pylori germ often causes no problems. But it can cause swelling and irritation, called inflammation, of the stomach's inner layer. When this happens, it can cause an ulcer.

  It's not clear how H. pylori infection spreads. It may go from person to person by close contact, such as kissing. People also can contract H. pylori through food and water.

- **Regular use of certain pain relievers.** Taking aspirin or nonsteroidal anti-inflammatory drugs (NSAIDs) over time can irritate or inflame the lining of the stomach and small intestine. These medicines include ibuprofen (Advil, Motrin IB, others), naproxen sodium (Aleve, Anaprox DS, others), ketoprofen and others. They do not include acetaminophen (Tylenol, others).

Risk factors

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If you take NSAIDs, the following factors may increase your risk of peptic ulcers:

- **Old age.** This includes people older than 60.
- **Prior peptic ulcer.** People who have had a peptic ulcer before have a higher risk of having another one.
- **NSAIDs use.** Taking high doses of NSAIDs or two or more NSAIDs increases the risk. So does taking NSAIDS with certain other medicines.

  These include other pain relievers, steroids, blood thinners, certain antidepressants called selective serotonin reuptake inhibitors (SSRIs) and medicines to treat the bone-thinning disease osteoporosis. These include alendronate (Fosamax, Binosto) and risedronate (Actonel, Atelvia).

Factors that don't cause peptic ulcers but can make them worse include:

- **Smoking.** This may increase the risk of peptic ulcers in people who are infected with H. pylori.
- **Drinking alcohol.** Alcohol can irritate and erode the mucous lining of the stomach. And it increases stomach acid.
- **Having untreated stress.**
- **Eating spicy foods.**

Complications

Untreated peptic ulcers can cause:

- **Bleeding in the stomach or duodenum.** Bleeding can be a slow blood loss that leads to too few red blood cells, called anemia. Or you can lose enough blood so that you need to be in a hospital or get blood from a donor. Severe blood loss may cause black or bloody vomit or black or bloody stools.

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- **A hole, called a perforation, in the stomach wall.** Peptic ulcers can eat a hole through the wall of your stomach or small intestine. This puts you at risk of infection of your abdomen, called peritonitis.
- **Blockage.** Peptic ulcers can keep food from going through the digestive tract. The blockage can make you feel full easily and cause you to vomit and lose weight.
- **Stomach cancer.** Studies have shown that people infected with H. pylori have an increased risk of stomach cancer.

Prevention

To help prevent peptic ulcers:

- **Take care with pain relievers.** If you often use NSAIDs, which can increase your risk of peptic ulcer, take steps to reduce your risk of stomach problems. For instance, take pain relievers with meals.

  Work with your healthcare professional to find the lowest dose that gives you pain relief. Don't drink alcohol with pain relievers. Together, they can increase your risk of stomach upset.

  If you need an NSAID, you also may need to take other medicines to help protect your stomach. These include antacids, proton pump inhibitors, acid blockers or cytoprotective agents.

  A class of NSAIDs called COX-2 inhibitors may be less likely to cause peptic ulcers. But these medicines may increase the risk of heart attack.
- **If you smoke, find a way to quit.** Quitting smoking can lower your risk of peptic ulcer. Talk with your healthcare professional for help with quitting.

By Mayo Clinic Staff

Peptic ulcer care at Mayo Clinic

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Diagnosis & treatment

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